LIBBY — PATIENT/CAREGIVER TRACK

demo-nsclc-egfr-l858r-post-osi-d3m0

Plain-language summary for patient and caregiver

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What this page is

Libby is an experimental tool that compiles published research about treatments that might apply to a particular cancer, then has five different "reviewers" with different priorities discuss the options. This page is the patient-friendly summary of what they came up with for a fictional case used to demonstrate the tool.

What we know about the cancer

This is a lung cancer (adenocarcinoma) that has spread beyond the lung (stage IV). The cancer carries two specific changes that doctors call "actionable" — meaning there are drugs that target them:

- A mutation in the **EGFR**gene called **L858R**.
- An amplification (extra copies) of the **MET**gene.

The patient was first treated with a pill called **osimertinib**that targets the EGFR mutation. It worked well for almost two years, then the cancer began growing again. The MET amplification is likely the reason the cancer became resistant.

What the patient said matters most

- They want a treatment they can take by mouth (a pill), not one that
- They specifically want to avoid:
  - **Severe nerve damage**(numbness, tingling, weakness — important for
  - **Heart problems**as a side effect.
  - **Hair loss.**
- They are open to enrolling in a clinical trial.
- On the question "do you want to push for the most effective treatment even

The options the board considered

Option 1 — savolitinib + osimertinib (preferably as part of a clinical trial)

The board recommended this. Four of five reviewers endorsed it; the fifth agreed with caveats.

What it is. Two pills taken at home. **Osimertinib**is the same drug the patient was already taking; **savolitinib**is added to it to block the MET pathway that probably caused the resistance. The combination has been studied in earlier-phase trials for exactly this situation (EGFR-mutant lung cancer that became resistant via MET amplification), and a larger phase-3 trial (called SAFFRON) is currently underway.

What the upside might look like. In the earlier trial (called TATTON), about 1 in 3 patients in this exact biomarker situation had measurable tumor shrinkage. The published numbers suggest the combination can re-establish disease control in a meaningful fraction of patients, but precise long-term benefit numbers are still being gathered.

The main risks. The most common side effects are liver-test changes (monitorable with blood draws), some leg swelling, and fatigue. None of these match the patient's specific vetoes.

Does it match what the patient wanted?Yes — all-oral, no infusions, no severe neuropathy / heart toxicity / hair loss in the published reports, and enrollable on a trial.

Did the board agree?Mostly yes. The "consensus" reviewer pointed out that this combination is not yet officially in the major guidelines as a recommended regimen, so the cleanest path to it is via clinical trial enrollment — which the patient was already open to.

Option 2 — amivantamab + lazertinib *(considered with caveats)*

The board considered this carefully but did NOT recommend it as the first choice.

What it is.A combination where one component (amivantamab) is given by intravenous infusion in the clinic, and the other (lazertinib) is a pill at home. Amivantamab is an antibody that targets both the EGFR and MET pathways.

What the upside might look like.A study (called CHRYSALIS-2) reported that about 36 of every 100 patients in a similar situation had measurable tumor shrinkage — a slightly higher rate than Option 1.

The main risks. Infusion reactions are common (sometimes severe), rash is nearly universal, and combination data show meaningful nerve-related side effects. The "conservative" reviewer specifically issued a **veto**because of the nerve-damage risk and the IV burden, both of which directly conflict with what the patient said they wanted to avoid.

Why it's still on the page. Two of the five reviewers (one focused on effect size, one focused on official guidelines) did endorse it. The board's rule is that a vetoed option is never silently dropped — the patient and clinician should see what was considered and rejected.

The board's bottom line. Option 2 is a real option, but only if Option 1 fails or proves intolerable. It violates the patient's stated preferences about IV time and nerve damage, and the modest extra effectiveness over Option 1 was not judged enough to override those preferences.

Option 3 — patritumab deruxtecan (HER3-DXd) *(considered with caveats)*

The board did not recommend this.

What it is. An antibody-drug conjugate given by IV infusion every three weeks. It carries a chemotherapy payload to cells that express a marker called HER3.

Why it's not recommended here. Hair loss is a frequent side effect, which the patient specifically said they wanted to avoid. The "advocate" reviewer removed it from active consideration on those grounds. There is also a small but real risk of lung inflammation (called ILD).

Why it's still on the page. For transparency. If the patient ever re-thinks the hair-loss veto, it's a real third-line option worth a conversation.

Questions to ask your oncologist

1. Am I a candidate for the **SAFFRON**or **SACHI**trial of savolitinib
2. If I'm not eligible for a trial, can savolitinib plus osimertinib still be

3. With Option 1, what's the monitoring plan for liver-function changes, and
4. If Option 1 stops working, what's the realistic next step — Option 2,
5. How current is the guideline status of these options? Anything new in the
6. What's the realistic time-to-response for Option 1, and how will we know
7. Are there any side effects I'd want to plan around given my work as an

Where to read more

- Clinician summary— the technical version of this page.
- Trial table— the trials Libby reviewed.
- Evidence list— the clinical and lab studies cited.
- Tumor-board transcript— what each reviewer actually said.
- Recommendations detail— full structured table.

Sources

The recommendations cited the following PubMed records and clinical-trial registrations:

- PMID 32679432 (TATTON expansion, savolitinib + osimertinib)
- PMID 36720074 (CHRYSALIS-2, amivantamab + lazertinib)
- PMID 37563559 (HERTHENA-Lung01, patritumab deruxtecan)
- NCT05261399 (SAFFRON phase-3 trial registry)